

1 ENGROSSED HOUSE
2 BILL NO. 3342

By: Williams of the House

3 and

4 Woods of the Senate
5
6

7 An Act relating to Medicaid audits; creating the
8 Oklahoma Medicaid Audit Bill of Rights Act; defining
9 terms; providing certain protections for health care
10 providers; providing for advance notice; providing
11 for specialty appropriate audit; limiting scope of
audits; directing for no allowance of extrapolation;
providing for appeals process; providing for
noncodification; providing for codification; and
providing an effective date.

12
13
14 BE IT ENACTED BY THE PEOPLE OF THE STATE OF OKLAHOMA:

15 SECTION 1. NEW LAW A new section of law not to be
16 codified in the Oklahoma Statutes reads as follows:

17 This act shall be known and may be cited as the "Oklahoma
18 Medicaid Audit Bill of Rights Act".

19 SECTION 2. NEW LAW A new section of law to be codified
20 in the Oklahoma Statutes as Section 5029.10 of Title 63, unless
21 there is created a duplication in numbering, reads as follows:

22 As used in this act:

23 1. "Audit" means an investigation or review of a claim
24 submitted by a health care provider if the investigation or review:

1 a. is conducted by an auditor, and

2 b. involves records, documents, or information other than
3 the filed claim;

4 2. "Auditor" means

5 a. an insurance company,

6 b. a third-party payor,

7 c. the Oklahoma Health Care Authority, or

8 d. an entity that represents a responsible party,
9 including a company or group that administers claims
10 services;

11 3. "Clerical or recordkeeping error" means a mistake in the
12 filed claim regarding a required document or record, including, but
13 not limited to:

14 a. a typographical error,

15 b. a scrivener's error, or

16 c. a computer error; and

17 4. "Health care provider" means a person who is licensed,
18 certified, or otherwise authorized by the laws of this state to
19 administer health care services to Medicaid patients.

20 SECTION 3. NEW LAW A new section of law to be codified
21 in the Oklahoma Statutes as Section 5029.11 of Title 63, unless
22 there is created a duplication in numbering, reads as follows:
23
24

1 A. Notwithstanding any other law, when an audit is conducted by
2 an auditor, the audit shall be conducted according to the following
3 bill of rights:

4 1. An auditor conducting the initial audit shall give the
5 health care provider notice of the audit at least one (1) week
6 before conducting the initial audit for each audit cycle;

7 2. An audit that involves the application of clinical or
8 professional judgment shall be conducted by or in consultation with
9 a health care provider of the same specialty as the health care
10 provider being audited;

11 3. A clerical or recordkeeping error shall not:

12 a. constitute fraud, or

13 b. be subject to criminal penalties without proof of
14 intent to commit fraud.

15 A claim arising pursuant to paragraph 3 of this subsection may
16 be subject to recoupment;

17 4. A finding of an overpayment or underpayment of a filed claim
18 may be a projection based on the number of patients served by the
19 health care provider having a similar diagnosis.

20 Recoupment of claims pursuant to this paragraph shall be based
21 on the actual overpayment unless the projection for overpayment or
22 underpayment is part of a settlement by the health care provider;

1 5. When an audit is for a specifically identified problem that
2 has been disclosed to the health care provider, the audit shall be
3 limited to a claim that is identified by a claim number;

4 6. For an audit other than that described in paragraph 5 of
5 this subsection, the audit shall be limited to the greater of:

6 a. fifty claims, or

7 b. twenty-five one-hundredths of one percent (0.25%) of
8 the number of claims billed by the health care
9 provider to the auditor in the previous calendar year;

10 7. If an audit reveals the necessity for a review of additional
11 claims, the audit shall be conducted by one of the following methods
12 at the discretion of the health care provider:

13 a. on-site,

14 b. electronically, or

15 c. by the same method as the initial audit;

16 8. Except for an audit initiated pursuant to paragraph 5 of
17 this subsection, an auditor shall not initiate an audit of a health
18 care provider more than two times in a calendar year;

19 9. A recoupment shall not be based on:

20 a. documentation requirements in addition to the
21 requirements for creating or maintaining documentation
22 prescribed by state law, rule, federal law or
23 regulation, or
24

b. a requirement that a health care provider perform professional duties prescribed by state law, rule, federal law, or regulation;

10. Recoupment shall only occur following the correction of a claim and shall be limited to amounts paid in excess of amounts payable under the corrected claim.

An auditor may recoup the entire overpaid claim if payment is issued for the corrected claim on the same date.

Following a notice of overpayment, a health care provider shall have at least sixty (60) days to file a corrected claim;

11. Approval of a health care service, health care provider, or patient eligibility upon adjudication of a claim shall not be reversed unless the health care provider obtained the adjudication by fraud or misrepresentation of claim elements;

12. Each health care provider shall be audited under the same standards and parameters as other similarly situated health care providers audited by the auditor;

13. A health care provider shall be allowed at least sixty (60) days following receipt of the preliminary audit report in which to produce documentation to address any discrepancy found during the audit;

14. The period covered by an audit shall not exceed twenty-four (24) months from the date the claim was submitted to or adjudicated by an auditor;

1 15. The preliminary audit report pursuant to paragraph 13 of
2 this subsection shall be delivered to a health care provider within
3 one hundred twenty (120) days after the conclusion of the audit.

4 A final audit report shall be delivered to the health care
5 provider within six (6) months after the receipt of the preliminary
6 audit report or receipt of the final appeal as provided for in this
7 subsection, whichever is later; and

8 16. Notwithstanding any other provision in this section, the
9 auditor conducting the audit shall not use the accounting practice
10 of extrapolation in calculating recoupments or penalties for audits.

11 B. A recoupment of any disputed funds shall only occur after
12 final internal disposition of the audit, including the appeals
13 process as described in subsection C of this section.

14 C. 1. An auditor that conducts an audit shall:

15 a. establish an appeals process under which a health care
16 provider may appeal an unfavorable preliminary audit
17 report to the auditor, and

18 b. provide a copy of the final audit report to the health
19 benefit plan sponsor after the completion of any
20 review process.

21 2. If following the appeal pursuant to subparagraph a of
22 paragraph 1 of this subsection the auditor finds that an unfavorable
23 audit report or any portion of the unfavorable audit report is
24 unsubstantiated, the auditor shall dismiss the audit report or the

1 unsubstantiated portion of the audit report without any further
2 proceedings.

3 D. The total amount of any recoupment on an audit shall be
4 refunded to the party responsible for payment of the claim.

5 SECTION 4. This act shall become effective November 1, 2026.

6 Passed the House of Representatives the 9th day of March, 2026.

7

8

Presiding Officer of the House
of Representatives

9

10

11 Passed the Senate the ____ day of _____, 2026.

12

13

Presiding Officer of the Senate

14

15

16

17

18

19

20

21

22

23

24